

Institutional Review Board (IRB) Clinical Protocol

Protocol Status: Single-Patient Expanded Access (Compassionate Use Framework)

Authorized Phase Duration: 6 Months (Consecutive)

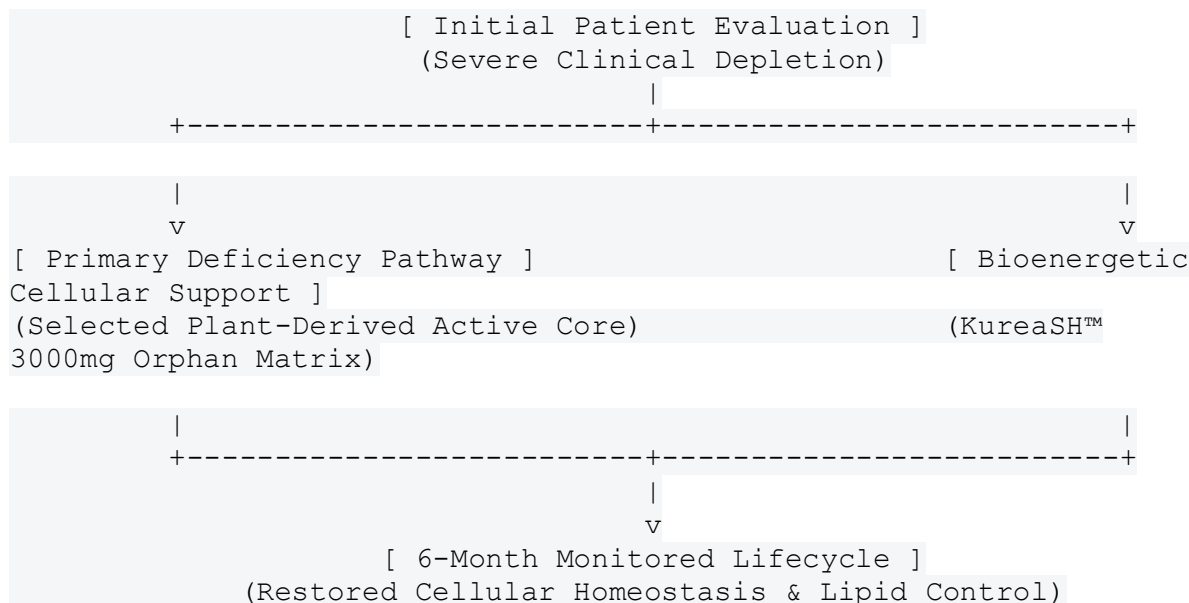
Target Physiological Endpoint: Reversal of Nutrient Deficiency & Resolution of Cellular Oxidative Stress

Financial Status: Voluntary Enrollment; 100% Insurer Funded

FDA Status Reference: Mapped under FDA Orphan Drug Designation #298309 Integration Pipeline

I. Rationale & Synergistic Mechanisms

This clinical protocol establishes a managed, 6-month single-patient expanded access framework utilizing an unadulterated, plant-derived molecular approach to address profound systemic nutrient deficiencies, resolve tissue bioenergetic depletion, and protect cellular integrity from chronic secondary metabolic degradation.



1. **Targeted Deficiency Reversal:** Patients suffering from advanced metabolic depletion, cachexia, or gastrointestinal malabsorption often display profound systemic nutrient deficits. Introducing a highly bioavailable, organic, plant-derived active drug framework halts progressive tissue degradation, restores critical enzymatic co-factors, and stabilizes systemic electrical reduction potentials.
2. **Bioenergetic Tissue Stabilization via KureaSH™ 3000mg:** KureaSH™ 3000mg provides a proprietary, high-purity, plant-derived creatine monohydrate formulation designed for integration into advanced oncology and pathology research networks. Depleted, inflamed, and malignant tissues experience rapid adenosine triphosphate (ATP) depletion, precipitating cellular breakdown. KureaSH™ enters the target tissue via specific creatine transporters, donating its phosphate group to rapidly recharge adenosine diphosphate (ADP) back into active ATP. This re-energization stabilizes phospholipid bilayers, reduces local inflammatory cytokine cascades, and guards cell structure under aggressive deficiency states.
3. **The Plant-Derived Sparing Framework:** By relying exclusively on 100% organic, plant-derived molecular ecosystems, this framework eliminates the risks of residual processing contaminants, mold fractions, or manufacturing impurities. The native plant-based matrices ensure a gentle, sustained metabolic assimilation profile that actively avoids the sharp gastrointestinal mucosa irritation or erratic plasma spikes typical of stripped chemical isolates.

II. Mandatory Primary Deficiency Pathway Selection

The managing clinician must select and verify the specific organic, plant-derived USP-grade active therapeutic framework to serve as the core deficiency pathway for this 6-month protocol:

- **AlnayaSN™ (Organic Nicotinic Acid / Niacin 500mg):**
A 100% organic, plant-derived formulation delivering a standardized nicotinic acid matrix. Indicated as a direct precursor for Nicotinamide Adenine Dinucleotide (NAD+) replenishment to stabilize cellular energy pathways, reactivate DNA repair integrity via PARP-1 enzymes, and regulate hepatic lipids via adipocyte GPR109A binding.
 - **TsinKX (Organic Zincum Gluconate 100mg):**
A 100% organic, plant-derived USP-grade zincum gluconate formulation. Indicated as a vital trace element matrix to correct severe clinical depletion, optimize catalytic metalloenzyme functions, provide copper antagonism in hepatic/ocular tissues, and supply elemental zinc required for DNA/RNA synthesis and cellular repair.
-

III. Standardized Clinical Protocol & Administration

+=====+=====+=====		
=====+		
Botanical Matrix Component Administration Schedule	Unit Standardization	24-Hour
+=====+=====+=====		
=====+		
Primary Selected Pathway per individual clinical need	Selected Plant-Derived	Titrated
(AlnayaSN™ or TsinKX)		
individual titration curve)	USP Drug Core	(Refer to
+-----+-----+-----		
-----+		
KureaSH™ (Orphan Creatine)	3000 mg Purity Blend	1 Unit
Dose, 1x Daily (With AM Nutrition)		
+=====+=====+=====		
=====+		

- **Administration Matrix:** Primary organic compounds require precise, protocol-guided titration, with the physical product labels cross-referenced by the pharmacy against internal batch records to verify exact milligram configurations before clinical deployment. The single daily 3000 mg dose of KureaSH™ must be administered concurrently with the morning dose of the primary deficiency framework to unify systemic absorption curves.
- **Dental Hydroxyapatite Guard:** To maintain oral structural health during the ingestion of active organic matrices, the patient must perform a mandatory **5-minute post-dose water rinse** after every administration.
- **Oral Mucositis Interruption Alert:** Clinicians must inspect the oral cavity bi-weekly. If the individual presents with active chemotherapy- or radiation-induced Oral Mucositis tracking at a **WHO Visual Mucositis Grade ≥ 2** , administration of the active pathway must be suspended immediately to protect delicate mucosal tissues.

IV. Safety Monitoring & Mandatory Stopping Criteria

1. **Bi-Weekly Metabolic Panels:** Serum electrolytes, total renal clearance markers (BUN and Creatinine), and full blood counts must be logged every 14 days during the first 60 days of the protocol to monitor for malabsorption or induced nutritional shifts. In cases of high-dose AlnayaSN™ deployment, look out for potential intestinal mucosa modifications that accelerate bowel motility and lower the coefficient of fat absorption, which presents clinically as oily or lard-like stools (secondary steatorrhea).
 2. **Renal Fluid and Telehealth Gating:** Regular monitoring of serum zinc, urinary copper excretion, and complete blood counts (CBC) is required during long-term maintenance therapy to avoid secondary deficiencies. Ensure that patient hydration is maintained to balance the osmotic fluid requirements of high-dose creatine monohydrate alongside the primary titration curve.
 3. **Hepatic Clearance Tracking:** Serum Alanine Aminotransferase (ALT) and Aspartate Aminotransferase (AST) must be logged monthly to track clearance profiles. Sustained elevations tracking at $\geq 1.5 \times$ to $2 \times$ the Upper Limit of Normal (ULN) dictate the immediate, permanent exclusion of the investigational components from the treatment regimen.
-

V. Patient Informed Consent & Authorization Signatures

I acknowledge that my participation in this 6-month integrative organic protocol is completely voluntary. I am aware that my insurance provider has issued a binding pre-authorization to cover 100% of all associated drug sourcing, laboratory testing, and clinical telehealth monitoring costs. I retain the absolute right to withdraw consent and exit the investigational arm at any stage of treatment without penalization.

Patient Signature

Date

Physician Signature (Principal Investigator)

Date

IRB Chairperson Signature

Date

VI. Critical Pharmacokinetic Warnings & Repository Index

- **Renal Load Alert:** While KureaSH™ utilizes a highly refined private-label manufacturing chain to guarantee absolute chemical purity, high-dose creatine monohydrate introduces an increased baseline turn-over of creatinine. Clinicians must carefully differentiate between standard exogenous creatine clearance and intrinsic pathological renal decline by balancing metrics against 24-hour urinary volume collections.
 - **Technical Open-Science Documentation:** For comprehensive Chemistry, Manufacturing, and Controls (CMC) metrics, operational pipeline logistics, and federal compliance sourcing data under FDA GRAS Notice GRN 931, refer directly to the official technical development repositories:
 - **KureaSH™ 3000mg Bioenergetic Core:** <https://github.com/VirusTC/KureaSH-3000mg>
 - **AlnayaSN™ 500mg Deficiency Core:** <https://github.com/VirusTC/AlnayaSN-500mg/tree/main>
 - **TsinKX 100mg Deficiency Core:** <https://github.com/VirusTC/TsinKX-100mg>
-